Chapter 48

Clinical Ethics for Obstetric Sonography

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Morality and Ethics

- Ethics defined as systematic reflection on and analysis of morality
- Morality concerns right and wrong conduct (what we ought or ought not do) and good and bad character (the kinds of persons we should become and the virtues we should cultivate in doing so).
- Morality reflects duties and values.

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Morality and Ethics (Cont.)

- Freedom and autonomy also integral to morality because values can be expressed
- All aspects of morality, duties, values, and rights are of importance in clinical ethics of sonography practice.

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Morality and Ethics (Cont.)

- Morality learned through personal experiences and family traditions and from normative behavior within communities, ethnic and racial groups, geographic regions
- Religions disagree about conduct and character, and religious ethics provide inadequate foundation for professional ethics in a culturally diverse society.

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Morality and Ethics (Cont.)

- Ethics is a discipline of study that seeks to articulate clear, consistent, coherent, practical guidelines for conduct and character.
- Ethics tries to answer key question “What is good?”
- To be applicable to medical context like sonography, ethics must transcend moral pluralism by offering approach with minimal ties to any substantive prior belief about moral conduct and character.

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History of Medical Ethics

- Prince Hammurabi of Babylon recorded responsibilities of health care providers in 1727 B.C.
- Hindu writers at about same time cautioned healers to treat patients with respect, gentleness, dignity.
- Fifth century B.C. Hippocrates cautioned his students, "*Primum non nocere,*" which famously means, "First, do no harm."

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History of Medical Ethics (Cont.)

- In ethics, these injunctions known as the ethical principle of nonmaleficence
- Hippocrates's teachings emphasized choosing treatments based on knowledge that would best benefit patients, treating patients like one would treat family member, upholding confidentiality, and practicing personal piety.

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History of Medical Ethics (Cont.)

- Thomas Percival (1740–1805) wrote treatise that substantially changed medical ethics.
- Previously, patient was someone who paid for treatment, but Percival redefined patient as anyone needing care.
- He also foresaw team approach in health care and public health.
- Percival emphasized patient care by all professionals and ordered competitive or professional interests be secondary to needs of patient.

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History of Medical Ethics (Cont.)

- Modern medical ethics were codified after Nuremberg trials, which judged atrocities done in medical experimentation by Nazi doctors.
- Judges in Nuremberg trial issued verdict that included a section on permissible human experimentation.
- That section, which became known as Nuremberg code, was incorporated into regulatory policy in United States.
- Nuremberg code emphasized individual rights and autonomy, and it has become key element of modern ethics.

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History of Medical Ethics (Cont.)

- Basic principles of medical ethics have been incorporated into research regulations, professional codes, and clinical practices throughout world.
- Ethical codes of different professional groups may differ slightly in definition and emphasis, but basic principles of autonomy, justice, beneficence, nonmaleficence, integrity, and respect for persons are universal.

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History of Medical Ethics (Cont.)

- *Code of Ethics for the Profession of Diagnostic Medical Sonography* adopted and maintained by Society of Diagnostic Medical Sonography

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Principles of Medical Ethics

Nonmaleficence

- Principle of nonmaleficence directs sonographer to not cause harm.
- Application of principle of nonmaleficence requires sonographer to obtain appropriate education and clinical skills to ensure competence in performing each examination required.

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Nonmaleficence (Cont.)

- Ensuring appropriate level of competence imposes rigorous standard of education and continuing education.
- Two problems result when obstetric sonographers do not maintain baseline level of competence in techniques and interpretation of sonographic imaging:
  - They may cause unnecessary harm to pregnant woman or fetal patient, for example, from mistaken impressions of fetal anomalies that in turn lead to unnecessary anxiety or testing.
  - May undermine informed consent process regarding management of pregnancy by reporting in an incomplete or inaccurate manner to physician who, in turn, reports misinformation to pregnant women.

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Nonmaleficence (Cont.)

- Sonographers need to be accountable for and participate in regular assessment and review of protocols, equipment, procedures, and results to ensure that patients are not harmed by outdated procedures or poorly functioning equipment.
- Appropriate oversight and approval of protocols by research or hospital committees contribute to patient safety.

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Nonmaleficence (Cont.)

- Protocols and diagnostic criteria should be established by peer review.
- Sonographers may also contribute to safety of patients by sharing with others and publishing peer-reviewed information about mistakes made or lessons learned.

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Nonmaleficence (Cont.)

- Sonographer must practice emergency procedures and strive to ensure patient safety in all procedures and circumstances.
- Sonographers must refrain from substance abuse or any activity that may alter their judgment or ability to provide safe and effective patient care.

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Nonmaleficence (Cont.)

- Because ultrasound energy poses theoretical risk to fetus, principle of nonmaleficence requires sonographers to perform all examinations with ALARA, energy exposure as low as reasonably possible, to achieve desired results.
- Sonographers need to read current medical literature to stay abreast of new developments related to patient safety.

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Beneficence

- Protections for patients and subjects based on ethical principle of nonmaleficence only partially explain what is in patient's interests because medicine, and therefore, sonography, seeks to benefit patients, not simply avoid harming them.

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Beneficence (Cont.)

- Use of obstetric ultrasound, like other medical interventions, must be justified by goal of seeking greater balance of clinical “goods” over “harms,” not simply avoiding harm to patient at all cost.
- This ethical principle is beneficence and is more comprehensive basis for ethics in sonography than nonmaleficence.

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Beneficence (Cont.)

- Goods and harms to be defined and balanced from rigorous clinical perspective
- Goods that obstetric sonography should seek for patients are preventing early or premature death (not preventing death at all costs); preventing and managing disease, injury, handicapping conditions; and alleviating unnecessary pain and suffering.

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Beneficence (Cont.)

- Pain and suffering are unnecessary and therefore represent clinical harms to be avoided when they do not contribute to seeking the good of the beneficence-based clinical judgment.
- Pain is physiologic phenomenon involving central nervous system processing of tissue damage.

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Beneficence (Cont.)

- Suffering is psychological phenomenon involving blocked intentions, plans, and projects.
- Pain often causes suffering, but one can suffer without being in pain.

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Beneficence (Cont.)

- Principle of beneficence obligates obstetric sonographer to seek greatest benefit in care of pregnant patients.
- Beneficence encourages sonographers to go beyond minimum standard protocol and to seek additional images and information if achievable and in best interests of patients.

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Beneficence (Cont.)

- Beneficence requires sonographers to focus on small comforts for patient, respecting their privacy and including their family upon request.
- Kindness and attention to small details minimize suffering caused by frustration or anger.
- Beneficence, like nonmaleficence, requires competency, knowledge, and excellent sonographic skills to ensure that patient and fetus receive greatest benefit from examination.

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Beneficence (Cont.)

- Fetal interests in sonography understood exclusively in terms of beneficence
- Principle explains moral (as distinct from legal) status of fetus as patient and generates ethical obligations owed by physicians and sonographers to fetus.

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Beneficence (Cont.)

- In technical language of beneficence, sonographer has beneficence-based obligations to fetal patient to protect and promote fetal interests and those of child it will become, as these are understood from rigorous clinical perspective.

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Beneficence (Cont.)

- Clinical good to be sought for fetal patient includes prevention of premature death, disease, handicapping conditions, and unnecessary future pain and suffering.
- Is therefore appropriate to refer to fetuses as patients, except when patient elects to terminate her pregnancy

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Beneficence (Cont.)

- In clinical practice, beneficence may have to be balanced against other ethical principles.
- Health professional’s duty of beneficence may suggest one course of action and patient may choose another.
- In these cases beneficence must be balanced by respect for person’s autonomy.

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Beneficence (Cont.)

- Principles of veracity and integrity may conflict with beneficence when truth-telling will cause undue stress and complications.

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Beneficence (Cont.)

- Principle of justice or fair distribution of benefits may conflict with beneficence for individual patients who need extra resources.
- Fortunately in most situations it is in patients' best interests to respect their autonomy, to tell the truth, and to distribute benefits justly.

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Autonomy

- Autonomy, or right to self-determination, has become key ethical principle.
- Respect for persons incorporates both respect for autonomy of individuals and requirement to protect those with diminished autonomy.

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Autonomy (Cont.)

- Patients, including pregnant women, have their own perspective on their interests that should be respected as much as the clinician’s perspective on patient’s interests.

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Autonomy (Cont.)

- Patient's perspective on her interests shaped by wide-ranging and sometimes idiosyncratic values and beliefs.
- Autonomy refers to person's capacity to formulate, express, and carry out value-based preferences.

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Autonomy (Cont.)

- Ethical principle of respect for autonomy obligates sonographer to acknowledge integrity of patient's values and beliefs and value-based preferences; to avoid interfering with expression or implementation of these preferences; and, when necessary, to assist in their expression and implementation.

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Autonomy (Cont.)

- Informed consent is autonomy-based right.
- Each health professional has autonomy-based obligations regarding informed consent process.

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Autonomy (Cont.)

- Process must include discourse about what sonography examinations can and cannot detect, sensitivity and frequency of false-negatives and false-positives of sonography techniques employed, and the difficult and sometimes uncertain interpretation of sonographic images.

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Autonomy (Cont.)

- In face of medical uncertainty about clinical good and harm of routine ultrasound, it is obligatory to inform pregnant patients about that uncertainty and to give them opportunity to make their own choices about how that uncertainty should be managed.

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Autonomy (Cont.)

- In routine examinations also important to inform woman of possibility of confronting anomaly that will lead her to decide whether to terminate pregnancy or take it to term

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Autonomy (Cont.)

- Genetic counselors, patient educators, sonographers, and physicians often work together to counsel women regarding their options and choices.

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Autonomy (Cont.)

- Sonographer respects patient's autonomy by providing detailed explanation of examination, including appropriate choices such as right to view screen or to determine gender of fetus.
- Respect for maternal autonomy dictates responding frankly to requests from pregnant woman for information about fetus's gender.

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Autonomy (Cont.)

- Pregnant woman should be made aware of uncertainties of ultrasound gender identification as part of disclosure process.
- Sonographer can use his or her own experience to help pregnant woman understand these uncertainties.

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Autonomy (Cont.)

- Second choice that may be presented during obstetric examinations is choice to view images.
- This choice related to the apparent bonding of pregnant women and their families to the fetus as result of seeing sonographic images
- Such bonding often enriches pregnancies that will be taken to term but can complicate decisions to terminate pregnancy.

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Autonomy (Cont.)

- Current ethical issue is nonmedical use of sonography, videotaping, or photography of “baby pictures.”
- Nothing intrinsically wrong with practice if it is side product of legitimate ultrasound examination
- However, when videotaping or photography is performed to generate revenue, practice trivializes medical sonography and may result in harm because problems that could be diagnosed might be missed.

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Autonomy (Cont.)

- Autonomy—enhancing strategy for woman to be allowed to insert vaginal probe herself to make experience more comfortable and less threatening
- Also sonographer's obligation to respect patient's right to refuse procedure

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Autonomy (Cont.)

- Maternal interests protected and promoted by both autonomy-based and beneficence-based obligations of sonographer to pregnant woman
- Fetuses incapable of having their own perspective on their interests because immaturity of their central nervous system renders them incapable of having requisite values or beliefs

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Autonomy (Cont.)

- Can be no autonomy-based obligations to fetus
- Pregnant woman also has beneficence-based obligations to fetal patient when pregnancy will be taken to term.
- She is expected to protect and promote fetal patient's interests and those of the child it will become.

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Autonomy (Cont.)

- When pregnant woman elects to have abortion, however, these obligations do not exist.
- Sonographer with moral objections to abortion should keep two things in mind:
  - Moral judgment and decision of pregnant woman to end her pregnancy should not be criticized or commented on in any way.
  - Her autonomy demands respect in form of sonographer's and physician's being neutral to her judgment and decision.

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Autonomy (Cont.)

- Sonographer is free to follow his or her conscience and to withdraw from further involvement with patients who elect abortion.
- Physicians should as matter of office policy respect this important matter of individual conscience on the part of the sonographer.

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Veracity and Integrity

- Telling truth is ethical principle that most sonographers have been taught from young age.
- Yet, vast majority of us will on occasion tell “white lies” in kindness or to escape unwanted consequences.
- Universal acceptance and even cultural preference in some countries for “white lies” evidence of difficulty adhering to principle of veracity.

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Veracity and Integrity (Cont.)

- Veracity means truthfulness.
- Integrity means adherence to moral and ethical principles.
- Integrity related to word *integrate* meaning *to bring together*
- In terms of honesty, integrity means there is no difference between what you think, what you say, what you do: they all come together in ethical behavior.

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Veracity and Integrity (Cont.)

- In medical care, patients properly rely for their protection on personal and professional integrity of their clinicians.
- Crucial aspect of that integrity on part of physicians is willingness to refer to specialists when limits of their own knowledge being approached.

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Veracity and Integrity (Cont.)

- Integrity should also be one of fundamental virtues of sonographers and a standard for judging professional character.
- Like other virtues, such as self-sacrifice and compassion, integrity directs sonographers to focus primarily on patient's interests as way to blunt mere self-interest.
- Sonographers must avoid conflicts of interest and situations that exploit others, create unreasonable expectations, or misrepresent information.

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Veracity and Integrity (Cont.)

- Veracity with respect to abilities and limitations absolutely essential among sonographers
- If practitioner asks sonographer to perform examination he or she is not competent to do, it is essential for sonographer to be truthful about his or her limitations to protect patient.
- Sonographer asked by patient or colleague must accurately represent his or her level of competence, education, credentials.

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Veracity and Integrity (Cont.)

- Premature disclosure of results of abnormal sonographic examination raises significant clinical ethical issues for sonographers.
- Sonographers justified in disclosing findings of normal anatomy directly to pregnant woman.
- When images reveal abnormal findings, sonographers must not act “dumb” or tell patient that they do not know what they are seeing.

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Veracity and Integrity (Cont.)

- Veracity upheld by telling patient in nonalarming way the procedure for diagnosis, that “multiple eyes need to look at some of images,” and that physician will determine results
- Disclosure of abnormal findings by sonographers inappropriate because it is not in best interest of patient

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Veracity and Integrity (Cont.)

- Sonographers must strive to supply patients and colleagues with complete and accurate information.
- Sonographer’s integrity is essential safeguard for patient’s autonomy.
- At times sonographer will need to become advocate, even vigorous advocate, for disclosure of information to patient.

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Veracity and Integrity (Cont.)

- In such cases sonographers must address their concerns not to patient but to practitioner involved.
- Failure to make patient disclosures undermines professional integrity and moral authority of health care professionals.

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Veracity and Integrity (Cont.)

- When sonographer disagrees with clinical judgment of his or her supervising physician, professional communication and discussion of matter need to occur.
- Best interests of health care team and patient are enhanced by such conversations.

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Justice

- Justice is ethical principle that requires fair distribution of benefits and burdens; injustice occurs when benefit to which person entitled is withheld or when burden is unfairly imposed.
- Justice means simply that sonographers must strive to treat all patients equally.

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Justice (Cont.)

- In practical terms, justice requires that translators be used when necessary to ensure adequate and appropriate communication with all patients.

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Justice (Cont.)

- Sonographers should strive to ensure that disabled patients have access to reasonable accommodations and pathways and that obese patients have comfortable chairs, gowns, and stretchers.
- Children, adults, and geriatric patients need to feel equally welcome and cared for within the sonography lab.

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Justice (Cont.)

- Justice served when protocols standardized
- Men and women with similar symptoms should receive similar tests and interventions.
- If a group denied services or asked to assume undue burden to obtain care provided to others, justice not being served

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Justice (Cont.)

- Justice and autonomy are ethical principles that determine timing of obstetric sonography examinations.
- Information obtained from sonogram enhances women’s choices.
- Is an injustice to provide this information to some women and not to others

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Justice (Cont.)

- For this reason, recommendations made that all women be offered risk assessment for anomalies during first trimester.
- Sonography results, such as risk for abnormality, are relevant to woman's decision about whether she will seek abortion.
- In pregnancies that will be taken to term, sonography also enhances pregnant woman's autonomy.

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Justice (Cont.)

- Principle of justice implies that health care professionals should act in accordance with best interests of community.
- As health care costs increase, insurance costs skyrocket, bankruptcy becomes associated with chronic illness, societal aspects of medical justice receiving more attention

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Justice (Cont.)

- Traditional focus of medical ethics has been individual patient.
- In some cases, costs and benefits of treating one patient may place undue burden on others.
- Individual ethical focus may be in conflict with society focus when individual uses disproportionate amount of health care without paying for it.

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Justice (Cont.)

- Forces others to pay for service, a burden society accepts if service considered essential
- As benefit of service decreases, as in experimental protocols, or cost of service increases, conflict grows.
- If resources used are not replaced, others may be deprived of similar services.
- Solution to such conflicts not clear politically, socially, or ethically

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Confidentiality of Findings

- Confidentiality concerns obligation of caregivers to protect clinical information about patients from unauthorized access
- Obligation of confidentiality derives from principles of beneficence (patients will be more forthcoming) and respect for autonomy (patient's privacy rights protected).

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Confidentiality of Findings (Cont.)

- Others, including pregnant woman's spouse and/or sex partner and family, should be understood as third parties to patient-sonographer relationship.
- Diagnostic information about woman's pregnancy confidential
- Can be justifiably disclosed to third parties *only* with pregnant woman's *explicit permission*

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Confidentiality of Findings (Cont.)

- Federal regulations, including Health Insurance Portability and Accountability Act (HIPAA), determine acceptable conditions for releasing confidential information.
- To prevent awkward situations, sonographers and supervising physicians should establish policies and procedures that reflect ethics of confidentiality.

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Confidentiality of Findings (Cont.)

- Ethics of confidentiality when pregnant woman less than age of 18 years should be same as when patient 18 years of age and older.

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Confidentiality of Findings (Cont.)

- The law, however, may complicate matters because pregnancy does not emancipate minor in every jurisdiction, and different jurisdictions give different levels of protection to privacy of physician-patient relationship when patient under age of 18 years.

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